

SECTION | 4

Challenges for Therapists

THERAPISTS HAVE PARTS: WHAT'S HARD IN THERAPY IS WHAT'S HARD FOR YOU

Just like clients, we therapists have parts and burdens that can interfere with therapy. Countertransference is an inevitable feature of all relationships. If we are oblivious about our parts and ignore their needs, we make it a problem. But we can make it an opportunity. Judgmental or fearful parts who pop up mark a spot where calm, curious inquiry will produce important knowledge about our parts' needs. Couples are never objectively "easy" or "difficult." For example, our dread of the next session with a withdrawn, uncommunicative couple is a prompt to find and unburden our own exiles. The following issues often cause countertransference reactions. They spring from the therapist's burdens and warrant exploration:

1. The couple's style of interacting reminds the therapist of their past.

- "My parents or caretakers acted this way."

Parts who had learning experiences with parents or other caretakers during childhood are likely to rouse when a couple exhibits similar behavior. Usually, we're talking about negative experiences from the past, but countertransference knows no bounds. If you particularly loved a grandparent and one member of the couple strikes you as similarly fun or benevolent, countertransference can be no less of an issue. Here, as always, our job is to be mindful of our parts' reactions to our clients' parts, due either to identification or dislike, and to use this information to clear the path and help our exiles unburden so we can be Self-led in our role as therapist.

2. The couple is navigating an affair or betrayal.

- "I was taught to disapprove of infidelity on moral grounds."
- "I've been hurt or someone I care about has been hurt by an infidelity."

An individual's response to infidelity (or betrayal) of any kind will depend on what it means to them, which depends on their history and culture. Affairs often mobilize protectors because they involve lies and signal at least some level of abandonment.

The meaning a person brings to their partner's infidelity is key. An affair might be taken to mean something shaming, like *I am not enough*, or it might evoke a judgment, like *You are immoral* or *You are bad*. Alternatively, it might highlight a real power differential and mean *I am helpless and cannot protect myself*. In any case, some therapists have strong feelings about infidelity because of their experience in childhood or in romantic relationships later, others will have learned to disapprove, and yet others will have developed strong feelings after witnessing someone else suffering. Our point is the same here as with all hot-button issues. When strong feelings arise, be mindful of your reactivity and help your own parts.

3. The couple presents with sexual problems.

- “I’m not trained for this.”
- “Some of my parts are embarrassed by this topic.”

We recommend raising the topic of sex even if your clients don't. Many couples struggle with their sexual relationship, but their parts are too embarrassed to mention it. Although you don't need to be a sex therapist to help a couple explore sex, you do need to feel comfortable talking about sex, exploring it, and listening well to clients. Therefore, knowing when to refer out is also important. If any countertransference issue feels overwhelming because of your experiences, you can get supervision or refer the couple for adjunctive treatment with a specialist. Also, where the body is concerned, you may need to involve medical treatment providers. That said, this is another topic that can mark important concerns in your system, which you will benefit by exploring.

4. Critical, shaming protectors in one or both partners attack the therapist.

- “Yikes! Do I deserve this?”
- “How can I get rid of this client?”
- “They’re hopeless.”
- “Their problems are endless.”
- “They’re not motivated to get better.”
- “I can’t be responsible for someone who acts this way.”

How can we get shamed? Let us count the ways. How do our protectors react? The list here illustrates some commonplace options. Whatever your vulnerabilities may be, a firefighter who is up to snuff will find them. If we are prepared, being shamed by a

client's firefighter is an opportunity because they show us what's going on inside us between a critic and exile, and what is being externalized in the direction of the couple. Preparation for shaming involves the usual advice (i.e., find your exiles and help them unburden). But even if all your exiles are perfectly unburdened, shaming is a hurtful behavior, and you can still have parts who take it personally. Therefore, prepare your protectors by reminding them that you are doing a job and that you don't need protecting. If you made a mistake or the client has a valid complaint, admit it and model a repair. A client's part who projects or picks a fight to distract from fear or emotional pain also offers a therapeutic opportunity.

5. Shamed exiles blend in one of the partners and then in the therapist.

- “This is too much for anyone to bear.”
- “They [the other partner] are too much.”

Emotional overwhelm is easily transmitted, highlighting the infantile beginning of empathy called emotional contagion. When one baby cries, other babies within earshot cry too. If the therapist has a young exile who feels shameful, identifies with the client's exile, and overwhelms the therapist, then the therapist's protectors will blame the client, believing the client is causing the therapist's pain.

6. A part of the therapist identifies with one partner, takes over internally (blends), and triangulates with the couple.

Couple therapy is inherently triangulating. Once we get triangulated with a couple, we may hear ourselves saying things like:

- “This partner is the problem. That one is balanced.”
- “This partner is too controlling; all they do is react.”
- “I like this one, but not that one.”

Additionally, we may find ourselves doing the following:

- Leaning toward one partner and away from the other.
- Feeling sorry for one partner.
- Identifying with one partner and then becoming protective.
- Taking sides.

In IFIO, we teach clinicians that couple therapists will unconsciously default to the triangles they negotiated as a child in their family of origin. Our job is to explore and resolve our family-of-origin triangles.

7. The therapist is working with a high-conflict couple.

In the language of IFIO, “high conflict” describes a couple presenting with protectors who feel extremely threatened and who will adopt any strategy to try to create intrapsychic safety. They are committed to their own stories, which usually involve blaming the other person and being right. These protectors have a negative effect on the nervous systems of both partners and the therapist. Because these parts focus exclusively on their idea of safety, they lose perspective quickly and frequently. If the therapist’s exiles respond with high anxiety, fear, cognitive overwhelm, or dissociation, the therapist’s protectors will say things like:

- “I can’t handle them.”
- “They should divorce.”
- “They are pathological.”
- “How can I fire them?”
- “I can’t help them.”

As always, our job is to know and help our exiles. If we need time to do that, or we are, for some other reason, not in a position to help a couple, we have a responsibility to address the fears and judgments of our parts and refer the couple with kindness.

8. Protectors react negatively to parts language or refuse to allow a U-turn.

- “I am helpless with this couple because they haven’t signed on for help and they don’t cooperate.” A client’s protective parts can refuse to budge for many reasons. Often, they feel misunderstood, disrespected, or invalidated. Some fear that parts language will threaten their control of the internal system, and some have had a negative experience in past therapy. Because these protectors can seem implacable, therapist parts who need to feel successful can become intolerant of them.

9. Dysregulated parts of the therapist emerge.

The IFIO motto is “we are them.” Therapists have parts and a Self. We have childhood wounds, relational histories, divorces, and so on. When the therapist’s burdens from unhealed relationship traumas activate with a couple in session, the therapist can transform

from a well-regulated, unblended guide to a frightened, overwhelmed child. No worries. With awareness, commitment, and practice, we can help our parts unblend consistently. Consultation is a good idea for some cases. If any of the following feelings or thoughts start repeating, it's time for your own U-turn:

- “These people overwhelm me.”
- “I am too anxious to be effective.”
- “It’s their fault.”
- “They remind me of my parents.”
- “Their trauma mirrors mine.”
- “I hate couple therapy.”
- “I’m incompetent.”

POTENTIALLY HOT TOPICS IN COUPLE THERAPY

Certain topics are more likely to bring up strong feelings in the therapist. These include:

- Sex and sexuality. For example:
 - o A polyamorous couple seeks help from a therapist who believes in monogamy.
 - o A couple in which one partner has had affairs and lied seeks help from a therapist who has not recovered from the infidelity of a partner or parent.
- Gender. For example:
 - o A couple with one or two trans partners seeks help from a cisgendered therapist whose beliefs are implicitly or explicitly biased regarding gender assignment at birth.
- Diversity and inclusion. For example:
 - o Couples of color seek help from a white therapist who is implicitly or explicitly racially biased.
 - o A biracial couple seeks help from a therapist of the same race as one partner and different from the other (or different from both partners), who is implicitly or explicitly racially biased.
- Religious, cultural, or political differences. For example:

- o A Catholic couple who is anti-abortion seeks help from a non-Catholic therapist who is pro-choice, and one or all of them are implicitly or explicitly biased toward people who hold different opinions.
- o A couple who disagrees about abortion seeks help from a therapist who agrees with one partner but not the other.
- o A Hasidic couple seeks help from a non-Hasidic therapist who believes that women need to be educated and equally empowered in relationships, or vice versa.
- o A couple who voted for Donald Trump seeks help from a therapist who supported Elizabeth Warren, or vice versa.

Regardless of the couple's presenting issue, and even when therapists feel reactive in response to a client, the IFIO protocol remains the same. This is also true of IFS, which is why therapists have used the IFS approach successfully with all kinds of populations across a wide variety of settings. When it comes to our personal experiences, we all inhabit specific, narrow perspectives. We also unconsciously generalize from our personal experiences, sometimes more accurately than other times. This is why the therapist's job in both IFIO and IFS is to maintain a continual U-turn stance, a kind of flowing assessment of our own level of blending and access to the Self's broad views. One of the hallmarks of the Self is having the ability to encompass multiple perspectives at once. When we do a U-turn and unblend (with the help of supervision, if necessary), we notice our responses to clients and we are more available to them.

That said, it behooves the therapist to be—or become—knowledgeable about the specifics of a couple's presenting problems. Clients always teach us about themselves, but it is not their responsibility to spend their time teaching us about issues we can learn on our own through reading and consultation. Some clients have specific requirements of a therapist (e.g., the therapist must be gay or African American), some prioritize their feelings about the therapist over any other consideration, and others steer clear of experts purposely because they want to guide their therapist's thinking on topics of importance to them. If a couple somehow ends up in your office and wants something you can't give, you can offer to get supervision and read up, or you can refer them.

Sometimes we want to involve other treatment providers. For example, it's wise for clients to get a professional assessment for medical issues, though it's also a good idea to explore if (and to what extent) parts might be involved with the problem. Expert advice can also be important to functional

issues, like certain sexual problems. A life-threatening problem, like some addictions and eating disorders, may also require medical expertise. But even if you refer one or both partners to another provider for their expertise, the couple may want to continue seeing you.

The following worksheet can help therapists explore and get in touch with any parts who contribute to personal patterns of reactivity in sessions with clients. IFIO therapists must keep countertransference in mind and notice if they are having strong reactions to one or both partners. Without this degree of mindfulness, a therapist's reactive protectors or frightened exiles will confuse the therapeutic relationship and block progress. In fact, when therapy stalls, explore countertransference first. It's likely to be the problem.

Therapist Worksheet**THERAPIST PARTS:
RESPONDING TO OUR OWN REACTIVITY**

All therapists get caught in their own dilemmas during therapy. As a rule, from the beginning of treatment, we ask therapists to be curious about their own responses to the partners in a couple. The following questions will help you be curious when your parts get reactive.

1. What sensations do you feel in your body? Describe their location, their intensity, and the movement of their energy.

2. What are your initial impulses (e.g., flight, fight, run, hide, numb, laugh, collapse, give up, protect)?

3. What do your parts say to you?

4. Identify and describe your protectors, either managers or firefighters.

5. Identify and describe your exiles.

6. Look back over your notes to identify any parts who react predictably and repeatedly. Would they be willing to meet your Self? What would help them be willing to unblend?
